

**BEDFORDSHIRE, LUTON AND MILTON KEYNES AREA PRESCRIBING
COMMITTEE (APC)**

**Contraception Prescribing Guidance for Primary
Care**

(May 2024)

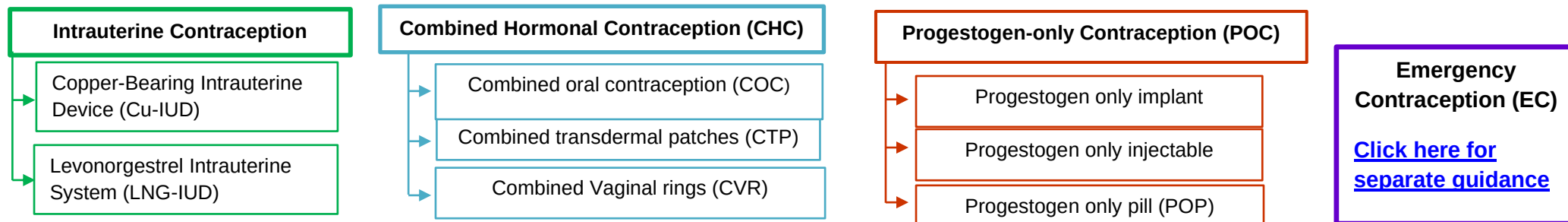
Approved by BLMK APC May 2024

Review date: May 2027

The following organisations contribute to and participate in the BLMK APC – Bedfordshire, Luton and Milton Keynes Integrated Care Board; Bedfordshire Hospitals NHS Foundation Trust; Cambridgeshire Community Services NHS Trust; Central and North West London NHS Foundation Trust; East London NHS Foundation Trust; Milton Keynes University Hospital NHS Foundation Trust

BLMK Contraception Prescribing Guidance

This guidance is based on best practice advice, given by the Faculty of Sexual Reproductive Health (FSRH), NICE Clinical Knowledge Summaries (CKS) and may vary from the individual drug SmPCs



Summary of key messages

This guideline should be read in conjunction with the individual [Summaries of Product Characteristics \(SmPCs\)](#); the relevant **FSRH guidance**, **FSRH UK Medical Eligibility criteria for contraceptive (UKMEC)** -see [FSRH UKMEC criteria \(2019\)](#), [CKS guidance](#) and the [eBNF](#)

UKMEC criteria should be applied to assess a woman's eligibility for use of IUD, CHC or POC.

- Women should be made aware that no method of contraception is 100% effective.
- Women requiring contraception should be given information about and offered a choice of all methods, including long-acting reversible contraception (LARC) methods.
- All currently available LARC methods (intrauterine devices, implants, and injectable contraceptives) are more cost effective than the combined oral contraceptive pill even at 1 year of use. IUDs and implants are more cost effective than injectable contraceptives.
- LARC methods are also the most reliable method of contraception and should be **1st Line choice** where possible. Information on effectiveness of the different methods can be found [here](#).
- Combined Oral Contraceptives (COC): choose a preparation with the oestrogen and progestogen combination which gives lowest associated VTE risk, good cycle control and reduced risk of side effects
- If a younger women aged 16 years of age or under requests contraception without parental consent, she should be assessed for her capacity to give informed consent to treatment. It should be documented in the clinical record whether or not [Fraser criteria is met](#). (NB this may have been documented previously).
- Nut and / or soya allergy:** Some formulations of both COC and POPs contain ingredients not suitable for women with nut and /or soya allergies - If applicable, refer to individuals SmPCs for excipient content – select a suitable preparation and continue to prescribe by **brand name**.
- Lactose** - All oral contraceptives contain lactose. See [FSRH advice](#) for further guidance on alternatives for vegans.
- All COCs** should be prescribe by **brand name**.
- POPs** can be prescribed generically but should be prescribed by brand in patients with nut and /or soya allergies
- Emergency contraception (EC)** - the copper coil is the most effective EC method: it should be offered to all women seeking emergency contraception (see separate [EC guidance](#)).

- History:** Take a full clinical history (medical, medication use, including OTC and non-prescribed, family, sexual, cervical smear, social, previous contraception use, lifestyle)
- Check:** BP, Weight and BMI
- Exclude:** STI, pregnancy if appropriate

- Determine women's preferences for contraception
- Exclude contraindications to chosen method using [UKMEC 2019](#)
- Promote additional use of barrier methods for protection against STI's
- If appropriate: Discuss Vasectomy as a potential method of contraception

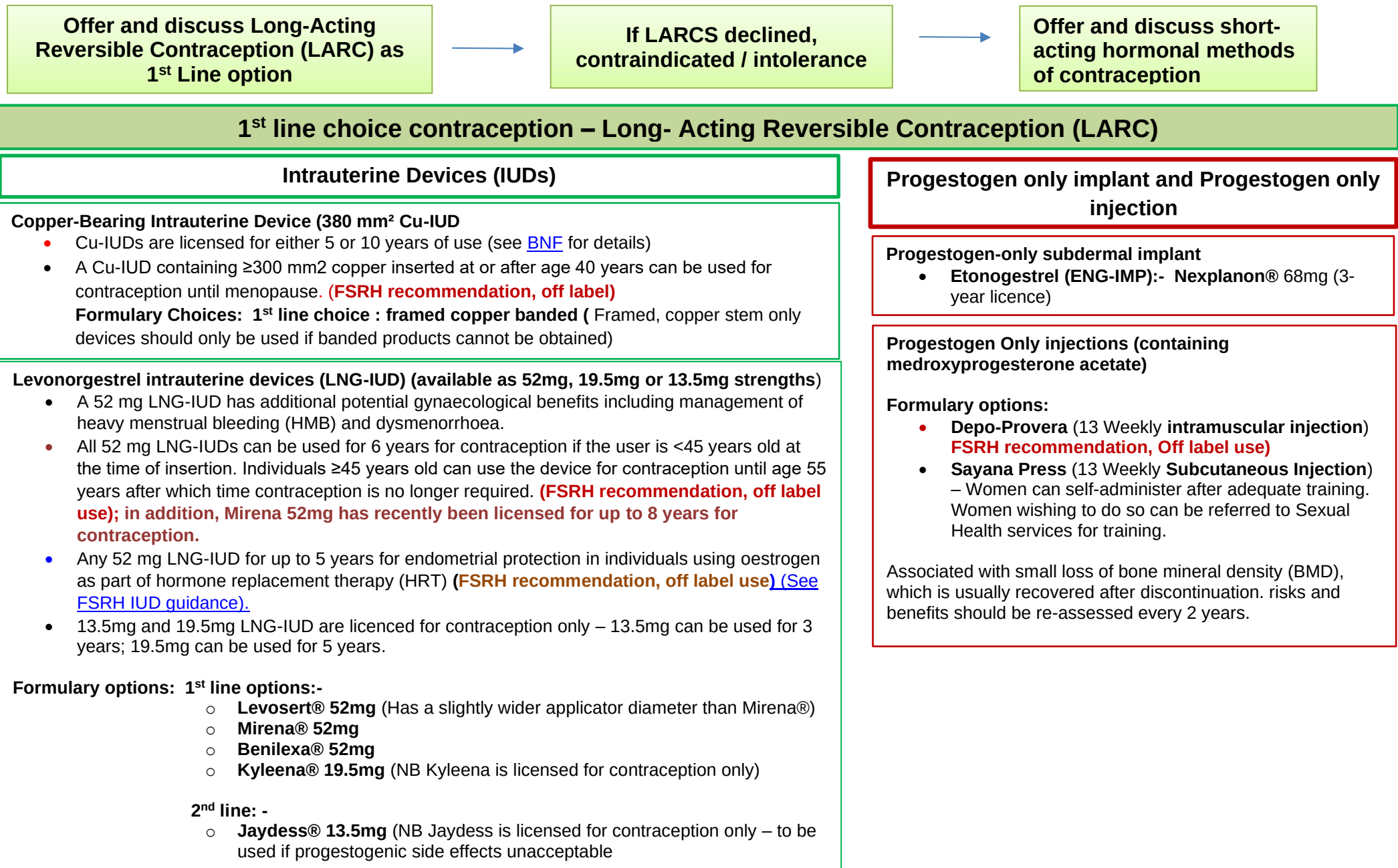
The UKMEC Categories

For each of the personal characteristics or medical conditions considered by the UKMEC a Category 1, 2, 3 or 4 is given. The definitions of the categories are given in Table 1.

Table 1: Definition of UKMEC categories

UKMEC	DEFINITION OF CATEGORY
Category 1	A condition for which there is no restriction for the use of the method
Category 2	A condition where the advantages of using the method generally outweigh the theoretical or proven risks
Category 3	A condition where the theoretical or proven risks usually outweigh the advantages of using the method. The provision of a method requires expert clinical judgement and/or referral to a specialist contraceptive provider, since use of the method is not usually recommended unless other more appropriate methods are not available or not acceptable
Category 4	A condition which represents an unacceptable health risk if the method is used

CHOICE OF CONTRACEPTION



Combined oral Contraception

- Advise women that taking the pill regularly at the same time of the day will aid adherence.

FSRH recommendations:

- Women should be given information about both standard and tailored CHC regimens to broaden contraceptive choice.
- Women should be advised that use of tailored CHC regimens is outside the manufacturer's licence but is supported by the Faculty of Sexual & Reproductive Healthcare (FSRH).
- Women should have access to clear information (either written or digital) to support tailored CHC use.

Tailored CHC Regimens

- Tailored regimens are as safe as and may be more effective for contraception than the standard 21/7 regimen and therefore should be actively promoted (NICE, **FSRH guidance, off-label use**). [See FSRH CHC guidance](#)
- Consider tailored regimens to broaden contraception choice and support compliance.
- Details of tailored regimens are available on CKS website - [CKS advice on tailored regimens](#)
- Traditional use of combined hormonal contraceptives (CHCs) is 21 days of active pills or one ring, or three patches, followed by a 7-day hormone-free interval (HFI).

Tailored (non-standard) CHC regimens:

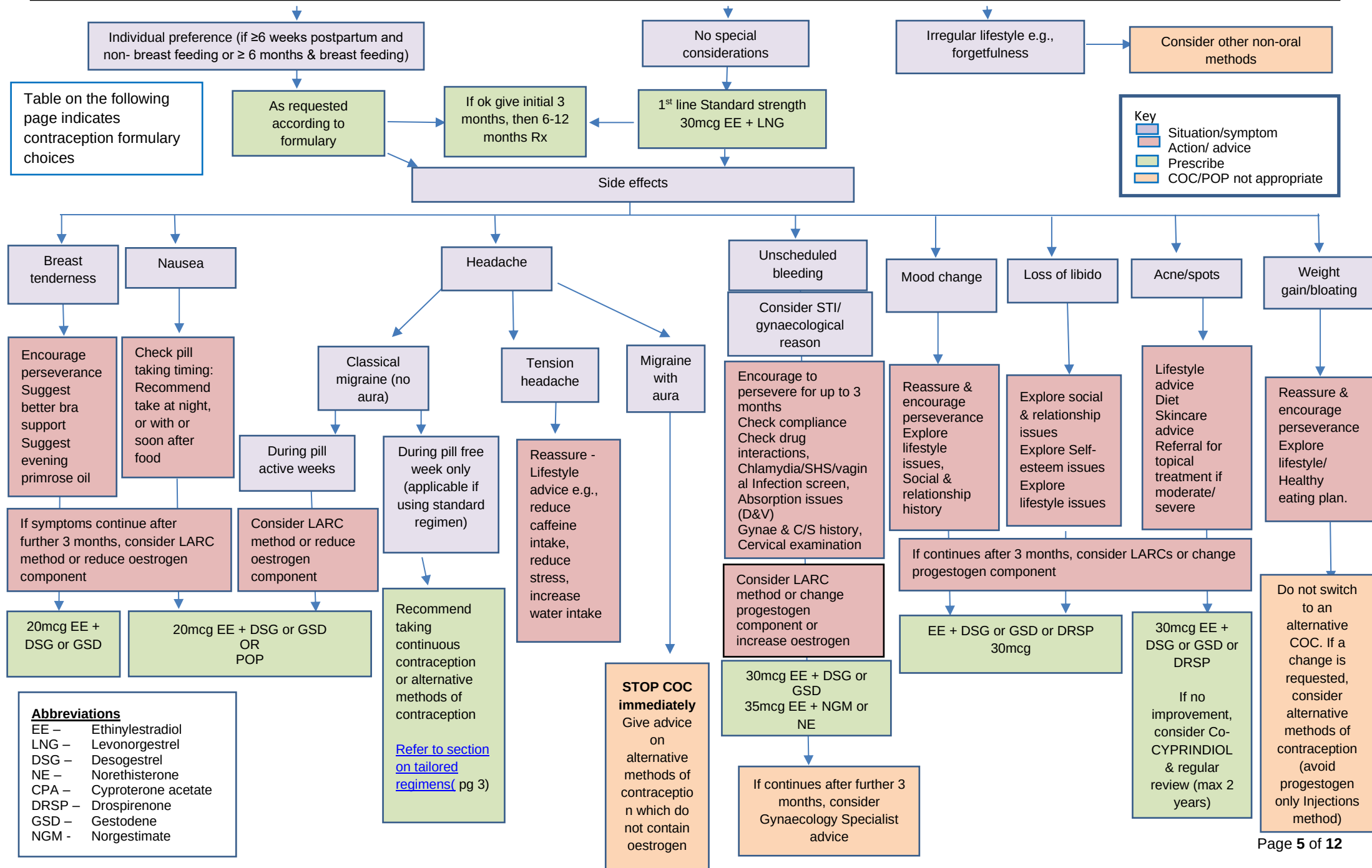
- Reduce the frequency of the Hormone free interval (HFI) (extended regimens), abolish the HFI (continuous regimens) and/or shorten the HFI
- May reduce or avoid HFI-associated symptoms.
- There is potentially a reduced risk of escape ovulation and resulting contraceptive failure with tailored regimens compared to traditional 21/7 regimens.

Follow-up and review of oral combined hormonal contraception:

- Arrange a follow up review 10–12 weeks after starting oral contraception.
- Re-check blood pressure and body mass index.
- Assess the woman for any new risk factors e.g. migraine that means her current method is no longer suitable ([UKMEC guidance](#))
- If the woman is experiencing troublesome side effects, give advice for managing side effects using [Flow chart 1](#) or change contraceptive method.
- Offer advice and information on considering LARCs, where there are no contraindications to use.
- Check compliance: Ensure the pill is being taken consistently and correctly and that the woman understands the missed or late pill taking and drug interaction advice.
- Review annually thereafter (or sooner if anything changes).

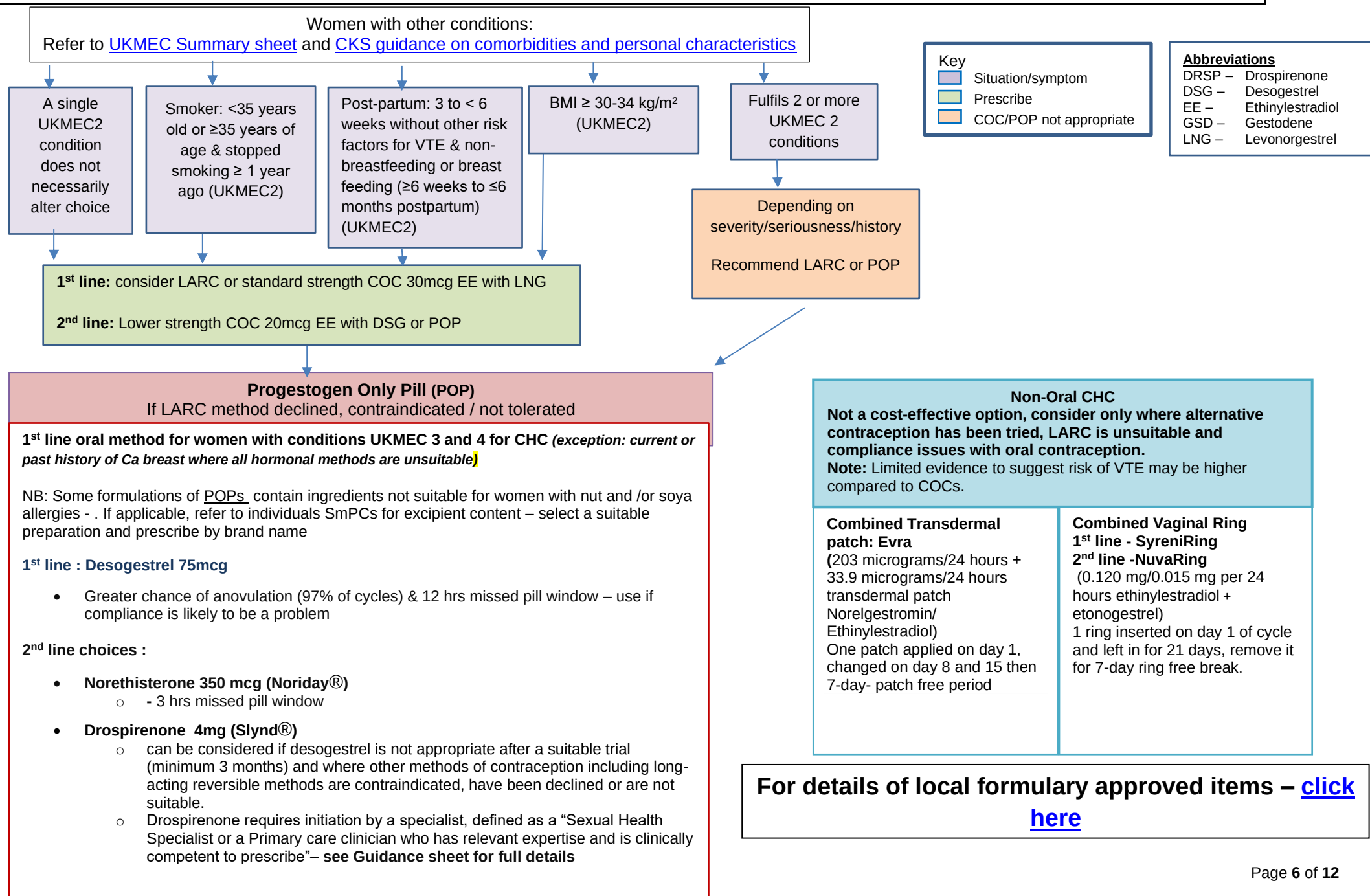
Combined Hormonal Contraception (CHC) - If LARC is declined or contraindicated or intolerance

Flow chart 1 – UKMEC category 1: A condition for which there is no restriction for the use of combined oral contraception (COC). Refer to [UKMEC summary sheet](#)



Combined Hormonal Contraception (CHC) continued

Flow chart 2 - UKMEC category 2: A condition where the advantages of using combined oral contraception generally outweigh the theoretical or proven risks



Additional Information

Missed pills

Combines Oral Contraceptive (COC) - see [Missed pill advice from CKS](#)
Progestogen Only Pill (POP) – See [Missed pill advice from CKS](#)

Drug interactions

Liver enzyme inducing drugs (e.g., some antibiotics, antiepileptics, antiretrovirals, St John's Wort, may reduce the efficacy of the COC and POP)

- Refer to the relevant drug's SmPC and eBNF for advice on managing the interaction and also for advice on if additional or alternative contraception methods should be used.
- Drug Interaction information is also available via SystemOne at point of prescribing.
- See also [FSRH advice on interacting drugs](#) that could reduce contraceptive effectiveness.
- If a woman is taking a teratogenic drug(s) or drug(s) with potential teratogenic effects – [see below](#).

Contraception for women using a known teratogenic drugs or drugs with potential teratogenic effects

NB This advice also applies to women whose partner is using a known teratogenic drugs or drugs with potential teratogenic effects.

- FSRH guidance advises that women of reproductive age who are taking known teratogenic drugs or drugs with potential teratogenic effects should always be advised to use highly effective contraception both during treatment and for the recommended timeframe after discontinuation to avoid unintended pregnancy.
- Women should be made aware that no method of contraception is 100% effective.
- Women should seek advice from a specialist, who will carry out a pregnancy risk assessment and provide evidence-based advice on the most suitable method for them.

If using a method of contraception which is considered 'highly effective'

- E.g. long-acting reversible contraceptives (LARC) copper intrauterine device (Cu-IUD), levonorgestrel intrauterine device (LNG-IUD), progestogen-only implant and male and female sterilisation, additional contraceptive precautions (e.g. condoms or a second effective contraceptive method) are not required.
- Use of condoms should be advised for protection against sexually transmitted infections.
- **NB women using progestogen only implant must not take any interacting drugs that could reduce contraceptive effectiveness.**

If using Combined hormonal Contraception (CHC) or Progestogen-only pill (POP) or Progestogen-only Injections

The typical use failure rate of combined hormonal contraception (CHC) and the progestogen-only pill (POP) is 9%; for progestogen-only injectables eg depot medroxyprogesterone acetate (DMPA) it is 6%.

- If pills, patches, vaginal rings or injectables are used then **an additional barrier contraception, such as condoms, are advised and regular pregnancy testing considered.**
 - **NB women using CHC or POP must not take any interacting drugs that could reduce contraceptive effectiveness.**
 - **Use of barrier methods, withdrawal and fertility awareness methods alone is not recommended.**
- Click here for the full [FSRH Clinical Effectiveness unit Statement on this subject](#).

Risk of Venous Thromboembolism

- Evidence from observational studies suggest that current use of CHC is associated with a 3- to 3.5-fold increase in VTE risk compared with non-use of CHC.
- Despite this increased risk, the number of VTE events in women using CHC remains very small. See figures below.
- VTE risk is lower when taking CHC than during pregnancy and the postpartum period.
- VTE risk is highest in the months immediately after initiation of CHC or when restarting after a break of at least 1 month. The risk then reduces over the first year of use and remains stable thereafter. The frequent stopping and restarting of CHC is discouraged.
- The European Medicines Agency (EMA) review published estimated figures for absolute risk of VTE in users of CHC evidence to suggest that the risk of VTE associated with different CHC was influenced by progestogen type:-
Estimated incidence (per 10 000 women per year of use)
 - Not using CHC and not pregnant: ~ 2
 - CHC with levonorgestrel, norgestimate or norethisterone: ~ 5-7
 - CHC with etonogestrel or norelgestromin: ~ 6-12
 - CHC with gestodene, desogestrel or drospirenone and co-pyridinol: ~ 9-12
- Due to limited evidence; it is difficult to compare the effect of Ethinylestradiol dose on VTE risk.
- Safety data for new formulations containing estradiol valerate, estradiol hemihydrate, dienogest, and nomegestrol acetate is limited. Preparations containing these formulations are non-formulary.
- Combined transdermal patch and vaginal ring: Long-term data on VTE risk with the patch or ring are limited and conflicting.
- The benefits of any CHC far outweigh the risk of serious side effects - prescribers and women should be aware of the major risk factors for thromboembolism, and of the key signs and symptoms.

Contraception in women over 40 years

In women (aged over 40 and ≤50 years of age, all intrauterine and hormonal methods of contraception can be considered provided there are no contraindications. (see [UKMEC, FSRH guidance](#))

- **Advise that hormone replacement therapy (HRT) does not provide contraception.**
- Advise that a woman is potentially fertile for 2 years after her last menstrual period if she is under 50 years of age, and for 1 year after her last period if she is 50 years of age or over. (NB fertility is difficult to determine if woman using HRT or contraception)
- Advise that, in general, women require contraception until the age of 55 years. From 55 years of age natural loss of fertility can be assumed for most women.
- All progestogen-only methods of contraception are safe to use alongside HRT.
- The FSRH IUD guideline supports the use of any 52 mg LNG-IUD for up to 5 years for endometrial protection in individuals using oestrogen as part of hormone replacement therapy (HRT). **(FSRH recommendation, off label use)** ([See FSRH IUD guidance](#))
- CHC can be used in eligible women ≤ 50 years of age as an alternative to HRT for relief of menopausal symptoms and prevention of loss of BMD provided there are no contraindications.
- COC with 30mcg EE and levonorgestrel or norethisterone should be considered 1st choice COC for women over 40 years due to risks of VTE, cardiovascular disease and stroke.
- When aged 50 or over advise women using CHC to stop and use a safer contraception method e.g. IUD, progestogen only implant, POP

Non-Contraceptive Benefits that can influence choice of contraception

Method	Health Benefits
Cu-IUD	May be associated with reduced risk of endometrial and cervical cancer
LNG-IUD	Reduced bleeding and pain associated with primary dysmenorrhoea, endometriosis and adenomyosis, management of HMB
Progestogen injection	Reduced bleeding or amenorrhoea is common May reduce pain associated with endometriosis
Progestogen implant	Improvement in dysmenorrhoea and endometriosis-associated pain
CHC	May increase BMD (depot medroxyprogesterone can reduce BMD) Improved bleeding – regular, lighter, less painful Significant reduction in risk of endometrial and ovarian cancer Reduced risk of colorectal cancer Improve acne, hirsutism and PCOS symptoms, reduce PMS symptoms
POP	Reduced menstrual pain

Formulary choices of Combined Oral Contraceptive (COC)

- Some formulations of COC pills contain ingredients not suitable for women with nut and /or soya allergies - If applicable, refer to individual SmPCs for excipient content and select a suitable preparation.

Standard strength preparations	Preferred brands (based on cost effectiveness)
Ethinylestradiol 30mcg / Levonorgestrel 150mcg	Rigevidon®, Maexeni®, Levest®
Lower oestrogen with alternative progestogen preparations	
Ethinylestradiol 20mcg / Desogestrel 150mcg	Bimizza®, Gedarel® (20/150)
Ethinylestradiol 20mcg / Gestodene 75mcg	Millinette® (20/75), Sunya®
Standard strength oestrogen with alternative progestogen preparations	
Ethinylestradiol 30mcg / Desogestrel 150mcg	Cimizt ®, Gedarel ® (30/150)
Ethinylestradiol 30mcg / Gestodene 75mcg	Millinette® 30/75, Katya ®
Ethinylestradiol 30mcg / Drospirenone 3mg	Yacella® (0.03mg/3mg), Dretine® (0.03mg/3mg)
Higher strength oestrogen	
Ethinylestradiol 35mcg / Norethisterone 500mcg	Brevinor®
Ethinylestradiol 35mcg / Norethisterone 1mg	Norimin® (1mg/0.035mg)
Ethinylestradiol 35mcg / Norgestimate 250mcg	Cilique®, Lizinna®

Summary Chart of U.S. Medical Eligibility Criteria for Contraceptive Use

Condition	Sub-Condition	Cu-IUD		LNG-IUD		Implant		DMPA		POP		CHC	
		I	C	I	C	I	C	I	C	I	C	I	C
Age		Menarche to <20 yrs: 2		Menarche to <20 yrs: 2		Menarche to <18 yrs: 1		Menarche to <18 yrs: 2		Menarche to <18 yrs: 1		Menarche to <40 yrs: 1	
		≥20 yrs: 1		≥20 yrs: 1		18-45 yrs: 1		18-45 yrs: 1		18-45 yrs: 1		≥40 yrs: 2	
				>45 yrs: 1		>45 yrs: 2		>45 yrs: 1					
Anatomical abnormalities	a) Distorted uterine cavity	4		4									
	b) Other abnormalities	2		2									
Anemias	a) Thalassemia	2		1		1		1		1		1	
	b) Sickle cell disease [‡]	2		1		1		1		1		2	
	c) Iron-deficiency anemia	2		1		1		1		1		1	
Benign ovarian tumors	(including cysts)	1		1		1		1		1		1	
Breast disease	a) Undiagnosed mass	1		2		2*		2*		2*		2*	
	b) Benign breast disease	1		1		1		1		1		1	
	c) Family history of cancer	1		1		1		1		1		1	
	d) Breast cancer [‡]												
	i) Current	1		4		4		4		4		4	
	ii) Past and no evidence of current disease for 5 years	1		3		3		3		3		3	
Breastfeeding	a) <21 days postpartum					2*		2*		2*		4*	
	b) 21 to <30 days postpartum												
	i) With other risk factors for VTE					2*		2*		2*		3*	
	ii) Without other risk factors for VTE					2*		2*		2*		3*	
	c) 30-42 days postpartum												
	i) With other risk factors for VTE					1*		1*		1*		3*	
	ii) Without other risk factors for VTE					1*		1*		1*		2*	
	d) >42 days postpartum					1*		1*		1*		2*	
Cervical cancer	Awaiting treatment	4	2	4	2	2		2		1		2	
Cervical ectropion		1		1		1		1		1		1	
Cervical intraepithelial neoplasia		1		2		2		2		1		2	
Cirrhosis	a) Mild (compensated)	1		1		1		1		1		1	
	b) Severe [‡] (decompensated)	1		3		3		3		3		4	
Cystic fibrosis [‡]		1*		1*		1*		2*		1*		1*	
Deep venous thrombosis (DVT)/Pulmonary embolism (PE)	a) History of DVT/PE, not receiving anticoagulant therapy												
	i) Higher risk for recurrent DVT/PE	1		2		2		2		2		4	
	ii) Lower risk for recurrent DVT/PE	1		2		2		2		2		3	
	b) Acute DVT/PE	2		2		2		2		2		4	
	c) DVT/PE and established anticoagulant therapy for at least 3 months												
	i) Higher risk for recurrent DVT/PE	2		2		2		2		2		4*	
	ii) Lower risk for recurrent DVT/PE	2		2		2		2		2		3*	
	d) Family history (first-degree relatives)	1		1		1		1		1		2	
	e) Major surgery												
	i) With prolonged immobilization	1		2		2		2		2		4	
	ii) Without prolonged immobilization	1		1		1		1		1		2	
	f) Minor surgery without immobilization	1		1		1		1		1		1	
Depressive disorders		1*		1*		1*		1*		1*		1*	

Key:	
1 No restriction (method can be used)	3 Theoretical or proven risks usually outweigh the advantages
2 Advantages generally outweigh theoretical or proven risks	4 Unacceptable health risk (method not to be used)

Condition	Sub-Condition	Cu-IUD		LNG-IUD		Implant		DMPA		POP		CHC	
		I	C	I	C	I	C	I	C	I	C	I	C
Diabetes	a) History of gestational disease	1		1		1		1		1		1	
	b) Nonvascular disease												
	i) Non-insulin dependent	1		2		2		2		2		2	
	ii) Insulin dependent	1		2		2		2		2		2	
	c) Nephropathy/retinopathy/neuropathy [‡]	1		2		2		3		2		3/4*	
	d) Other vascular disease or diabetes of >20 years' duration [‡]	1		2		2		3		2		3/4*	
Dysmenorrhea	Severe	2		1		1		1		1		1	
Endometrial cancer [‡]		4	2	4	2	1		1		1		1	
Endometrial hyperplasia		1		1		1		1		1		1	
Endometriosis		2		1		1		1		1		1	
Epilepsy [‡]	(see also Drug Interactions)	1		1		1*		1*		1*		1*	
Gallbladder disease	a) Symptomatic												
	i) Treated by cholecystectomy	1		2		2		2		2		2	
	ii) Medically treated	1		2		2		2		2		3	
	iii) Current	1		2		2		2		2		3	
	b) Asymptomatic	1		2		2		2		2		2	
	a) Suspected GTD (immediate postevacuation)												
	i) Uterine size first trimester	1*		1*		1*		1*		1*		1*	
	ii) Uterine size second trimester	2*		2*		1*		1*		1*		1*	
	b) Confirmed GTD												
	i) Undetectable/non-pregnant β-hCG levels	1*	1*	1*	1*	1*		1*		1*		1*	
Gestational trophoblastic disease [‡]	ii) Decreasing β-hCG levels	2*	1*	2*	1*	1*		1*		1*		1*	
	iii) Persistently elevated β-hCG levels or malignant disease, with no evidence or suspicion of intrauterine disease	2*	1*	2*	1*	1*		1*		1*		1*	
	iv) Persistently elevated β-hCG levels or malignant disease, with evidence or suspicion of intrauterine disease	4*	2*	4*	2*	1*		1*		1*		1*	
	a) Nonmigraine (mild or severe)	1		1		1		1		1		1*	
	b) Migraine												
	i) Without aura (includes menstrual migraine)	1		1		1		1		1		2*	
	ii) With aura	1		1		1		1		1		4*	
	a) Restrictive procedures	1		1		1		1		1		1	
	b) Malabsorptive procedures	1		1		1		1		3		COCs: 3	
												P/R: 1	
History of bariatric surgery [‡]	a) Pregnancy related	1		1		1		1		1		2	
	b) Past COC related	1		2		2		2		2		3	
History of cholestasis													
History of high blood pressure during pregnancy		1		1		1		1		1		2	
History of Pelvic surgery		1		1		1		1		1		1	
HIV	a) High risk for HIV	1*	1*	1*	1*	1		1		1		1	
	b) HIV infection					1*		1*		1*		1*	
	i) Clinically well receiving ARV therapy	1	1	1	1	If on treatment, see Drug Interactions							
	ii) Not clinically well or not receiving ARV therapy [‡]	2	1	2	1	If on treatment, see Drug Interactions							

Abbreviations: ARV = antiretroviral; C=continuation of contraceptive method; CHC=combined hormonal contraception (pill, patch, and, ring); COC=combined oral contraceptive; Cu-IUD=copper-containing intrauterine device; DMPA = depot medroxyprogesterone acetate; I=initiation of contraceptive method; LNG-IUD=levonorgestrel-releasing intrauterine device; NA=not applicable; POP=progestin-only pill; P/R=patch/ring; SSRI=selective serotonin reuptake inhibitor; ‡ Condition that exposes a woman to increased risk as a result of pregnancy. *Please see the complete guidance for a clarification to this classification: https://www.cdc.gov/reproductivehealth/contraception/contraception_guidance.htm.

Summary Chart of U.S. Medical Eligibility Criteria for Contraceptive Use

Condition	Sub-Condition	Cu-IUD		LNG-IUD		Implant		DMPA		POP		CHC	
		I	C	I	C	I	C	I	C	I	C	I	C
Hypertension	a) Adequately controlled hypertension	1*		1*		1*		2*		1*		3*	
	b) Elevated blood pressure levels (properly taken measurements)												
	i) Systolic 140-159 or diastolic 90-99	1*		1*		1*		2*		1*		3*	
	ii) Systolic ≥160 or diastolic ≥100 [†]	1*		2*		2*		3*		2*		4*	
	c) Vascular disease	1*		2*		2*		3*		2*		4*	
Inflammatory bowel disease	(Ulcerative colitis, Crohn's disease)	1		1		1		2		2		2/3*	
Ischemic heart disease [‡]	Current and history of	1		2	3	2	3	3		2	3	4	
Known thrombogenic mutations [‡]		1*		2*		2*		2*		2*		4*	
Liver tumors	a) Benign												
	i) Focal nodular hyperplasia	1		2		2		2		2		2	
	ii) Hepatocellular adenoma [‡]	1		3		3		3		3		4	
	b) Malignant [‡] (hepatoma)	1		3		3		3		3		4	
Malaria		1		1		1		1		1		1	
Multiple risk factors for atherosclerotic cardiovascular disease	(e.g., older age, smoking, diabetes, hypertension, low HDL, high LDL, or high triglyceride levels)	1		2		2*		3*		2*		3/4*	
Multiple sclerosis	a) With prolonged immobility	1		1		1		2		1		3	
	b) Without prolonged immobility	1		1		1		2		1		1	
Obesity	a) Body mass index (BMI) ≥30 kg/m ²	1		1		1		1		1		2	
	b) Menarche to <18 years and BMI ≥ 30 kg/m ²	1		1		1		2		1		2	
Ovarian cancer [‡]		1		1		1		1		1		1	
Parity	a) Nulliparous	2		2		1		1		1		1	
	b) Parous	1		1		1		1		1		1	
Past ectopic pregnancy		1		1		1		1		2		1	
Pelvic inflammatory disease	a) Past												
	i) With subsequent pregnancy	1	1	1	1	1	1	1	1	1	1	1	1
	ii) Without subsequent pregnancy	2	2	2	2	1	1	1	1	1	1	1	1
	b) Current	4	2*	4	2*	1	1	1	1	1	1	1	1
Peripartum cardiomyopathy [‡]	a) Normal or mildly impaired cardiac function												
	i) <6 months	2		2		1		1		1		4	
	ii) ≥6 months	2		2		1		1		1		3	
	b) Moderately or severely impaired cardiac function	2		2		2		2		2		4	
Postabortion	a) First trimester	1*		1*		1*		1*		1*		1*	
	b) Second trimester	2*		2*		1*		1*		1*		1*	
	c) Immediate postseptic abortion	4		4		1*		1*		1*		1*	
Postpartum (nonbreastfeeding women)	a) <21 days					1		1		1		4	
	b) 21 days to 42 days												
	i) With other risk factors for VTE					1		1		1		3*	
	ii) Without other risk factors for VTE					1		1		1		2	
	c) >42 days					1		1		1		1	
Postpartum (in breastfeeding or non-breastfeeding women, including cesarean delivery)	a) <10 minutes after delivery of the placenta												
	i) Breastfeeding	1*		2*									
	ii) Nonbreastfeeding	1*		1*									
	b) 10 minutes after delivery of the placenta to <4 weeks	2*		2*									
	c) ≥4 weeks	1*		1*									
	d) Postpartum sepsis	4		4									

Condition	Sub-Condition	Cu-IUD		LNG-IUD		Implant		DMPA		POP		CHC	
		I	C	I	C	I	C	I	C	I	C	I	C
Pregnancy		4*		4*		NA*		NA*		NA*		NA*	
Rheumatoid arthritis	a) On immunosuppressive therapy	2	1	2	1	1		2/3*		1		2	
	b) Not on immunosuppressive therapy	1		1		1		2		1		2	
Schistosomiasis	a) Uncomplicated	1		1		1		1		1		1	
	b) Fibrosis of the liver [‡]	1		1		1		1		1		1	
Sexually transmitted diseases (STDs)	a) Current purulent cervicitis or chlamydial infection or gonococcal infection	4	2*	4	2*	1		1		1		1	
	b) Vaginitis (including trichomonas vaginalis and bacterial vaginosis)	2	2	2	2	1		1		1		1	
	c) Other factors relating to STDs	2*	2	2*	2	1		1		1		1	
Smoking	a) Age <35	1		1		1		1		1		2	
	b) Age ≥35, <15 cigarettes/day	1		1		1		1		1		3	
	c) Age ≥35, ≥15 cigarettes/day	1		1		1		1		1		4	
Solid organ transplantation [‡]	a) Complicated	3	2	3	2	2		2		2		4	
	b) Uncomplicated	2		2		2		2		2		2*	
Stroke [‡]	History of cerebrovascular accident	1		2		2	3	3		2	3	4	
Superficial venous disorders	a) Varicose veins	1		1		1		1		1		1	
	b) Superficial venous thrombosis (acute or history)	1		1		1		1		1		3*	
Systemic lupus erythematosus [‡]	a) Positive (or unknown) antiphospholipid antibodies	1*	1*	3*		3*		3*	3*	3*		4*	
	b) Severe thrombocytopenia	3*	2*	2*		2*		3*	2*	2*		2*	
	c) Immunosuppressive therapy	2*	1*	2*		2*		2*	2*	2*		2*	
	d) None of the above	1*	1*	2*		2*		2*	2*	2*		2*	
Thyroid disorders	Simple goiter/ hyperthyroid/hypothyroid	1		1		1		1		1		1	
Tuberculosis [‡] (see also Drug Interactions)	a) Nonpelvic	1	1	1	1	1*		1*		1*		1*	
	b) Pelvic	4	3	4	3	1*		1*		1*		1*	
Unexplained vaginal bleeding	(suspicious for serious condition) before evaluation	4*	2*	4*	2*	3*		3*		2*		2*	
Uterine fibroids		2		2		1		1		1		1	
Valvular heart disease	a) Uncomplicated	1		1		1		1		1		2	
	b) Complicated [‡]	1		1		1		1		1		4	
Vaginal bleeding patterns	a) Irregular pattern without heavy bleeding	1		1		2		2		2		1	
	b) Heavy or prolonged bleeding	2*		1*	2*	2*		2*		2*		1*	
Viral hepatitis	a) Acute or flare	1		1		1		1		1		3/4*	2
	b) Carrier/Chronic	1		1		1		1		1		1	1
Drug Interactions													
Antiretrovirals used for prevention (PrEP) or treatment of HIV	Fosamprenavir (FPV)	1/2*	1*	1/2*	1*	2*		2*		2*		3*	
	All other ARVs are 1 or 2 for all methods.												
Anticonvulsant therapy	a) Certain anticonvulsants (phenytoin, carbamazepine, barbiturates, primidone, topiramate, oxcarbazepine)	1		1		2*		1*		3*		3*	
	b) Lamotrigine	1		1		1		1		1		3*	
Antimicrobial therapy	a) Broad spectrum antibiotics	1		1		1		1		1		1	
	b) Antifungals	1		1		1		1		1		1	
	c) Antiparasitics	1		1		1		1		1		1	
	d) Rifampin or rifabutin therapy	1		1		2*		1*		3*		3*	
SSRIs		1		1		1		1		1		1	
St. John's wort		1		1		2		1		2		2	

Updated in 2020. This summary sheet only contains a subset of the recommendations from the U.S. MEC. For complete guidance, see: https://www.cdc.gov/reproductivehealth/contraception/contraception_guidance.htm. Most contraceptive methods do not protect against sexually transmitted diseases (STDs). Consistent and correct use of the male latex condom reduces the risk of STDs and HIV.

Missed Combined Oral Contraception pills

If **ONE** pill has been missed (48 – 72 hours since last pill in current packet or 24- 48 hours late starting first pill in new pack)

Continuing contraceptive cover

- The missed pill should be taken as soon as it is remembered
- The remaining pills should be continued at the usual time

Minimising the risk of pregnancy

Emergency contraception (EC) is not usually required but may need to be considered if pills have been missed earlier in the packet or in the last week of the previous packet

Minimising the risk of pregnancy

- *If pills are missed in the 1st week (Pills 1-7):* EC should be considered if unprotected sex occurred in the pill-free interval or in the first week of pill taking
- *If pills are missed in the second week (pills 8-14):* No indication for EC if pills in the preceding 7 days have been taken consistently and correctly, provided the pills thereafter are taken correctly and additional contraceptive precautions used
- *If pills are missed in the third week (pills 15-21):* **OMIT THE PILL-FREE INTERVAL** by finishing the pills in the current pack (or discarding any placebo tablets) and starting a new pack the next day

If **TWO** or **MORE** pills have been missed (>72 since last pill in current packet or >48 hours late starting first pill in new packet)

Continuing contraceptive cover

- The most recent pill missed should be taken as soon as possible.
- The remaining pills should be continued at the usual time.
- Condoms should be used or sex avoided until seven consecutive active pills have been taken. This advice may be over-cautious in the second and third weeks, but the advice is a back-up in the event that further pills are missed.

Missed Progesterone Only Pills

Northisterone POPs

>3 hours late (>27 hours since the last pill was taken)

Desogestrel POPs

>12 hours late (>36 hours since the last pill was taken)

- The missed pill should be taken as soon as remembered. If more than one pill has been missed, only one pill should be taken.
- The next pill should be taken at the usual time. This may mean that two pills are taken in one day.
- Additional contraceptive precautions (condoms or avoidance of sex) are advised for 2 days (48 hours) after restarting the POP.
- Emergency contraception is indicated if unprotected sexual intercourse occurred after the missed pill and within 48 hours of restarting the POP.

Approaching Menopause

Choice of Contraception: Methods that can be used without restriction

- Barrier methods
- Copper intra-uterine devices (IUD)
- Levonorgestrel releasing Intrauterine system (IUS)
- Progesterone only pill, progesterone only implant
- Progesterone only injections can be used until age of 50
- Combined hormonal contraception is not contraindicated by age alone but factors like smoking and migraine history must be considered. If suitable, a pill containing 20 mcg of ethinylestradiol is a reasonable first choice.

Non-contraceptive Benefits can influence the choice of contraceptive:

- Vasomotor symptoms (hot flushes): combined hormonal contraception may reduce symptoms.
- Osteoporosis: Combined hormonal contraception may increase bone mineral density. Depot medoxyprogesterone acetate can reduce BMD.
- Menstrual pain, bleeding, and irregularity: combined hormonal contraception may reduce symptoms. Progestogen-only methods may reduce pain
- Heavy menstrual bleeding: The LNG-IUS reduces menstrual bleeding and can cause amenorrhoea.

Advise Women that Hormone Replacement Therapy does not provide contraception

Stopping Contraception

If using a non-hormonal method of contraception: Continue until:

- 1y of amenorrhoea >50 years of age Or 2 years of amenorrhoea <50 years of age
- If a women continues to menstruate >55years, advise contraception use until 1 year of amenorrhoea has passed.

If using a hormonal method of contraception:

If woman wishes to stop contraception aged <50 years: advise to switch to a non-hormonal method and wait until 2 years of amenorrhoea (3 years if switching from progestogen only injections)

Combined hormonal contraception and progestogen only injections

- Continue until aged 50, then switch to a non-hormonal method OR switch to one of the following: POP, Progestogen only implant or LNG-IUS
- Follow advice for chosen method

POP, Progestogen only implant or LNG-IUS

- Continue until aged 55
- if woman still not amenorrhoeic at the age of 55 continue until 1 year of amenorrhoea
- If amenorrhoeic and aged > 50, arrange confirmation of menopause (two FSH readings taken 6 weeks apart and results of both tests are >30) and continue contraception for another year

Switching or Starting Methods of Contraception

This quick guide advises on effective starting or switching between contraceptive methods to reduce the risk of unplanned pregnancy. It summarises information from existing FSRH guidance. This guide should be used in conjunction with clinical judgment in individual situation.

Key to Symbols:



= start method immediately; condoms (or abstinence) not required



= start method, additional condom use (number of days given)



= do not start method

QS = consider 'quick starting' a method

Key to abbreviations:

CHC – combined hormonal contraception (pills, ring, patch)
Cu-IUD – copper intrauterine device
DMPA – depot medroxyprogesterone acetate
EC – emergency contraception
HFI – hormone free interval
LNG-IUS – levonorgestrel-releasing intrauterine system
QS – quick starting
POP – progestogen-only pill
PT – pregnancy test
UPSI – unprotected sexual intercourse

Quick starting

Quick starting describes starting contraception at the time a woman presents (rather than waiting for the next menstrual cycle). This may be outside the licence of some products. Use of a 'bridging' contraceptive method may be required until pregnancy can be excluded and the woman's preferred contraceptive method started.

For further information see [FSRH guideline Quick Starting Contraception \(2017\)](#)

CURRENT SITUATION	Starting:					
No recent hormonal contraception:	CHC	POP	DMPA	Implant	LNG-IUS	Cu-IUD
Day 1-5 of natural cycle (regardless of UPSI)						
After day 5 of natural cycle, if no UPSI since LMP OR Amenorrhoea, if no UPSI for >3 weeks and PT negative	 for 7 days	 for 2 days	 for 7 days	 for 7 days	 for 7 days	
After day 5 of natural cycle, if UPSI since LMP OR Amenorrhoea, if UPSI in last 3 weeks and PT negative Consider the need for EC	QS  for 7 days PT at 3 weeks	QS  for 2 days PT at 3 weeks	QS another bridging method or QS  for 7 days PT at 3 weeks	QS  for 7 days PT at 3 weeks	X QS another bridging method	if all UPSI in last 5 days or if within 5 days of earliest likely ovulation  If not X QS another bridging method

CURRENT METHOD	Switching to:					
Combined hormonal contraception: (if taken correctly)	CHC	POP	DMPA	Implant	LNG-IUS	Cu-IUD
CHC Week 2-3 OR Day 1-2 of HFI					 If day 2 of HFI follow advice for day 3-7 below	
CHC Week 1 OR Day 3-7 of HFI <u>AND</u> No UPSI since start of HFI	 for 7 days	 for 2 days	 for 7 days	 for 7 days	 for 7 days	
CHC Week 1 OR Day 3-7 of HFI <u>AND</u> UPSI since start of HFI	Continue CHC until taken for 7 consecutive days then advice as for weeks 2 or 3					

Progestogen only methods:	CHC	POP	DMPA	Implant	LNG-IUS	Cu-IUD
Implant (in situ for <3 years)	☺	☺	☺	☺	☺	☺
DMPA <14 weeks since last injection	☺	☺	☺	☺	☺	☺
DMPA >14 weeks since last injection AND No UPSI since 14 weeks	 for 7 days	 for 2 days	 for 7 days	 for 7 days	 for 7 days	☺
DMPA >14 weeks since last injection AND UPSI since 14 weeks <i>Consider the need for a PT and EC</i>	QS  for 7 days PT at 3 weeks	QS  for 2 days PT at 3 weeks	QS another bridging method or QS  7 days PT at 3 weeks	QS  for 7 days PT at 3 weeks	Unless able to exclude pregnancy, X QS another bridging method	If only UPSI since expiry was in last 5 days ☺ If UPSI since expiry was > 5 days ago, and unable to exclude pregnancy X QS another bridging method

Traditional POP (if taken correctly)	 for 7 days		 for 7 days	 for 7 days	 for 7 days	
Desogestrel POP (if taken correctly)						
Intra-uterine contraception:	CHC	POP	DMPA	Implant	LNG-IUS	Cu-IUD
LNG-IUS (in date) If no UPSI in last 7 days	 for 7 days	 for 2 days	 for 7 days	 for 7 days		
LNG-IUS (in date) UPSI in last 7 days	Retain LNG-IUS for 7 days after starting	Retain LNG-IUS for 7 days after starting	Retain LNG-IUS for 7 days after starting	Retain LNG-IUS for 7 days after starting	Ideally abstain for 7 days prior to change in case new device can't be inserted	
Cu-IUD day 1-5						
Cu-IUD after day 5 AND No UPSI in the last 7 days	 for 7 days	 for 2 days	 for 7 days	 for 7 days	 for 7 days	
Cu-IUD after day 5 AND UPSI in last 7 days	Retain Cu-IUD for 7 days after starting	Retain Cu-IUD for 7 days after starting	Retain Cu-IUD for 7 days after starting	Retain Cu-IUD for 7 days after starting	Abstain for 7 days prior to change, then  for 7 days	Ideally abstain for 7 days prior to change in case new device can't be inserted then 

CURRENT SITUATION	Starting:					
Incorrect use of existing hormonal method or method has expired	CHC	POP	DMPA	Implant	LNG-IUS	Cu-IUD
Incorrect use of existing hormonal method or method has expired IE no UPSI for >3 weeks and PT negative	 for 7 days	 for 2 days	 for 7 days	 for 7 days	 for 7 days	
Incorrect use of existing hormonal method or method has expired IE UPSI in last 3 weeks AND PT negative Consider the need for EC	QS  for 7 days PT at 3 weeks	QS  for 2 days PT at 3 weeks	QS another bridging method or QS  for 7 days PT at 3 weeks	QS  for 7 days PT at 3 weeks	X QS another bridging method (Please note for Mirena IUS only: Women who were under the age of 45 years at the time of Mirena IUS insertion and who present for replacement of the device between 5 and 7 years after insertion may have immediate replacement if a pregnancy test is negative and another pregnancy test is advised no sooner than 3 weeks after the last episode of UPSI)	If all UPSI in last 5 days or If within 5 days of earliest likely ovulation  If not X QS another bridging method

CURRENT SITUATION	Starting:					
After emergency contraception	CHC	POP	DMPA	Implant	LNG-IUS	Cu-IUD
Immediately after ulipristal acetate (ellaOne®)	Do not start any hormonal contraception for 5 days after ellaOne				×	If all UPSI within 5 days or if less than 5 days after earliest likely date of ovulation  If not ×
5 days after ulipristal acetate (ellaOne®)	QS  for 7 days PT at 3 weeks	QS  for 2 days PT at 3 weeks	QS another bridging method. or QS  for 7 days PT at 3 weeks	QS  for 7 days PT at 3 weeks	× Consider QS another bridging method	If all UPSI within 5 days or if less than 5 days after earliest likely date of ovulation  If not × and consider QS another bridging method

CURRENT SITUATION	Starting:					
After emergency contraception:	CHC	POP	DMPA	Implant	LNG-IUS	Cu-IUD
Levonorgestrel 1500mcg	QS  for 7 days PT at 3 weeks	QS  for 2 days PT at 3 weeks	QS another bridging method <u>or</u> QS  for 7 days PT at 3 weeks	QS  for 7 days PT at 3 weeks	X Consider QS another bridging method	If all UPSI within 5 days or if less than 5 days after earliest likely date of ovulation  If not X and QS another bridging method

The CEU is grateful to Dr Prachi Rajgarhia and the clinical staff of the Chalmers Centre in NHS Lothian, Edinburgh for developing and testing this document. The CEU also acknowledges the work of Dr Jayne Kavanagh and Dr Melissa Gardner from London in the original design of the document.

Other colleagues in UK centres also sent in versions of their own switching documents which were helpful comparisons.

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Emergency Contraception (EC) after unprotected sexual intercourse (UPSI) or failure of contraception

This BLMK guidance aims to summarise the key points to consider with regards to choice of emergency contraception. For comprehensive guidance, on the most appropriate choice, depending on individual circumstances, please refer to the following resources:-

- **Faculty of Sexual Reproductive Healthcare (FSRH) - Emergency Contraception - [full guidance](#)**
- **FSRH decision making algorithms:-**
[Algorithm 1: Decision-making Algorithm For Copper Intrauterine Device \(Cu-IUD\) vs Oral EC](#)
[Algorithm 2: Levonorgestrel EC \(LNG-EC\) vs Ulipristal Acetate EC \(UPA-EC\)](#)
- **Clinical Knowledge Summaries (CKS) - Emergency Contraception - [full guidance](#)**
- **Ardens templates - GPs can access the Ardens templates via SystemOne.**

Carry out a detailed initial assessment:-

- Check to see if EC is needed and that a pregnancy is reasonably excluded.
- Enquire if the woman is currently using hormonal contraception.
- Ask about timing of last menstrual period (LMP), average cycle length, timing and number of episodes of unprotected sexual intercourse (UPSI) since LMP.
- Determine if UPSI is likely to have occurred at a high risk time e.g. if woman is currently ≤ 5 days after earliest date of ovulation.
- The choice of EC should be decided by taking additional factors into consideration, such as the [UK Medical Eligibility Criteria](#), the woman's preference, likely time of ovulation in menstrual cycle, current medication, e.g. liver enzyme inducing medications, body mass index/weight, length of time since UPSI and whether post partum or breastfeeding.
- If working from a Patient Group Direction (PGD), check the woman meets the inclusion criteria specified within the PGD.

1st line choice: 380 mm² Copper IUD (Cu-IUD)

Points to note:

- **Cu-IUD is the most effective method of EC** - offer to **all** women provided the criteria for insertion is met and is acceptable.
- Cu-IUD can be inserted for EC within 5 days (120 hours) after the first unprotected sexual intercourse (UPSI) in a cycle or within 5 days of the earliest estimated date of ovulation, whichever is later.
- Emergency coil fitting appointments can be accessed at local sexual health clinics – [Click here](#) for contact details for local centres
- Offer oral EC at the time of referral in case a Cu-IUD cannot be inserted or woman changes her mind.
- Cu-IUD provides effective ongoing contraception

The following organisations contribute to and participate in the BLMK APC – Bedfordshire, Luton and Milton Keynes Integrated Care Board; Bedfordshire Hospitals NHS Foundation Trust; Cambridgeshire Community Services NHS Trust; Central and North West London NHS Foundation Trust; East London NHS Foundation Trust; Milton Keynes University Hospital NHS Foundation Trust

2nd line choice : Oral emergency contraception

The choice of oral EC is influenced by various factors such as length of time since UPSI, timing of UPSI in relations to likely timing of ovulation within a cycle, woman's body mass index / weight, any contraindications, any concurrent medications e.g. liver enzyme -inducing drugs, whether woman is post partum or breastfeeding.

General Points to consider:

- **Ulipristal acetate has been demonstrated to be more effective than levonorgestrel.**
- The use of oral EC post ovulation is ineffective.
- Ulipristal acetate and levonorgestrel can be prescribed on a FP10, obtained from a sexual health clinic, obtained from a community pharmacy (via a PGD or purchased over the counter (OTC)).
- **Length of time since UPSI** should be taken into account when considering a suitable choice of oral EC.

Oral EC options if :
Up to 72 hours after
UPSI

Ulipristal acetate 30mg (EllaOne) OR Levonorgestrel 1.5mg

(to aid decision making, consult [FSRH decision making algorithm](#) or [CKS guidance](#))

Oral EC options if:
Between 72 hours
to 120 hrs after
UPSI

Ulipristal acetate 30mg (EllaOne)

NB:-

- If other EC methods are unsuitable or declined, Levonorgestrel may be used **up to 96 hours after UPSI (FSRH guidance, off-label use).**
- **Levonorgestrel is ineffective if taken more than 96 hours after UPSI.**

Additional information

- Ulipristal acetate and levonorgestrel can be used once or more in a menstrual cycle, however:-
 - **Do not use** ulipristal acetate if previous use of levonorgestrel in the preceding/last 7 days.
 - **Do not use** levonorgestrel if previous use of ulipristal acetate in the last 5 days.
- If progestogen (in any form e.g. the progestogen only pill, HRT etc) has been taken 7 days prior to UPSI:- the effectiveness of ulipristal acetate could theoretically be reduced and levonorgestrel should be considered instead,
- Effectiveness of ulipristal acetate could be reduced if a woman takes progestogen in the 5 days after taking ulipristal EC.
- Effectiveness of both ulipristal acetate and levonorgestrel is reduced with concurrent use of enzyme-inducing medication or within 28-days of stopping enzyme inducer:- In such cases, refer to a specialist centre asap - if Cu-IUD not suitable, second line option is **double dose (3mg) levonorgestrel (FSRH guidance, off-label use)**. **Ulipristal acetate is not recommended.**
- Ulipristal acetate is not suitable for use by women who have severe asthma controlled by oral glucocorticoids.
- The effectiveness of levonorgestrel could be reduced if the woman has a BMI greater than 26 kg/m² or body weight greater than 70 kg - a dose of 1.5mg levonorgestrel should **NOT** be used - a **double dose (3mg) levonorgestrel is recommended (FSRH guidance, off-label use)**
- If time since UPSI is over 120 hours see [CKS guidance on over 120 hours](#); refer to the Sexual Health services for an EC-IUD assessment.
- For breast feeding mothers:- refer to [FSRH guidance](#) (section 12, page 19).

Additional advice

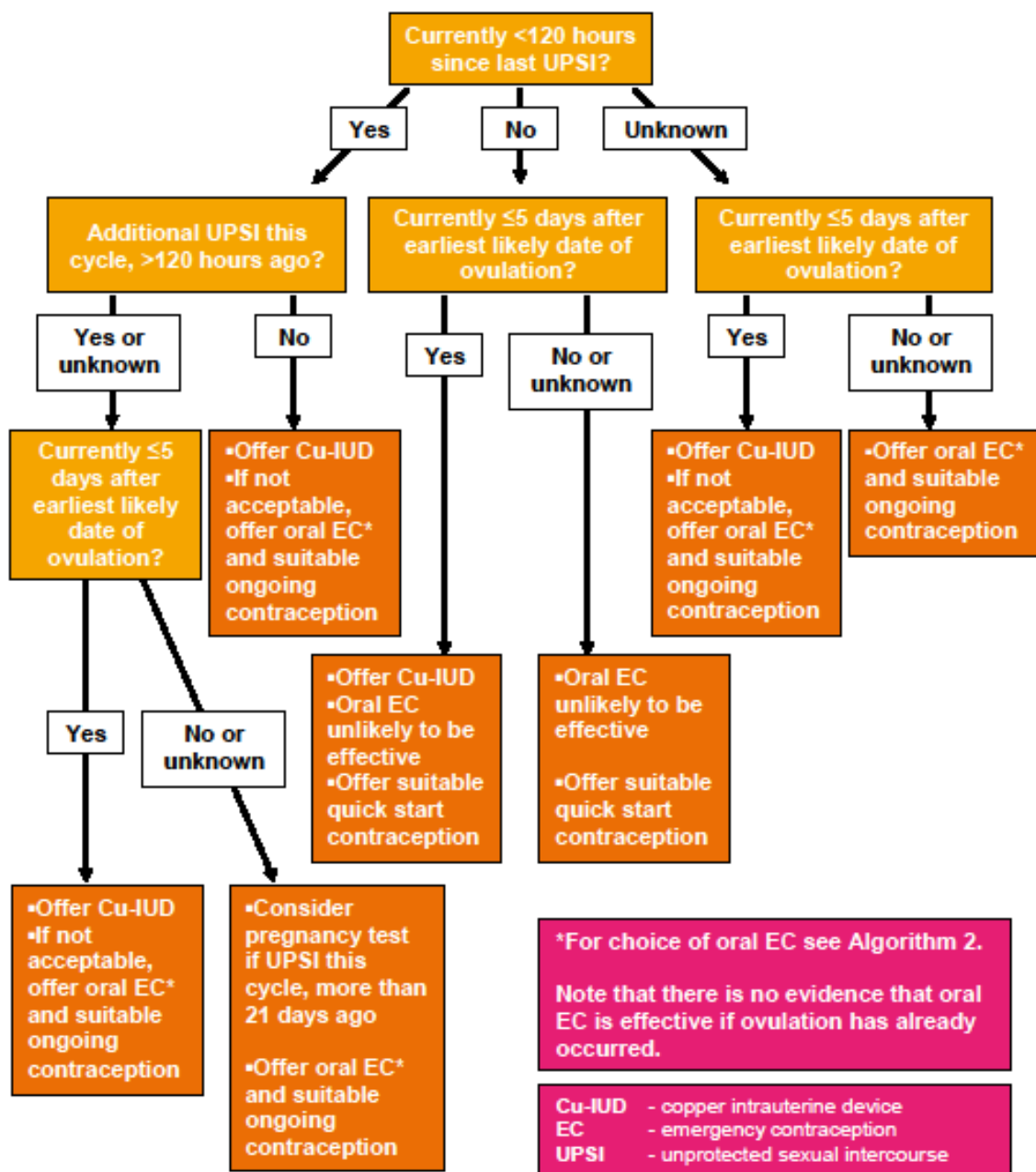
- If the woman vomits up to 3 hours after taking an oral EC, a second dose should be taken as soon as possible.
- Explain that after taking oral EC, a woman will continue to be at risk of pregnancy from UPSI later in the cycle as ovulation has been delayed.
- Offer suitable contraception - the **FSRH** advises that hormonal contraception should begin immediately after levonorgestrel and, five days after ulipristal acetate
- Advise to use additional contraception (such as a condom) or avoid sexual intercourse until the chosen contraception choice becomes effective—See [CKS continuing or starting regular contraception after emergency contraception](#) for full details.
- Advise a follow up pregnancy test after 3 weeks following the UPSI.
- Risk assess for safeguarding issues and for sexually transmitted infections (STIs). Recommend attendance to Sexual Health Services.

Algorithm 1: Decision-making Algorithm For Copper Intrauterine Device (Cu-IUD)



Decision-making Algorithms for Emergency Contraception

Algorithm 1: Decision-making Algorithm for Emergency Contraception (EC): Copper Intrauterine Device (Cu-IUD) vs Oral EC



Algorithm 2: Levonorgestrel EC (LNG-EC) vs Ulipristal Acetate EC (UPA-EC)



**Algorithm 2: Decision-making Algorithm for Oral Emergency Contraception (EC):
Levonorgestrel EC (LNG-EC) vs Ulipristal Acetate EC (UPA-EC)**

